PRE-CLASS PREP

Elimination

Know Before You Go

Read this before class — it will make the lecture click faster.

Big picture: Today covers both urinary and bowel elimination — assessment terms, common alterations, devices and interventions, and key conditions. Elimination is deeply personal; nurses prioritize privacy, dignity, and early identification of problems.

URINARY ASSESSMENT TERMS

Urgency — immediate desire to void

Dysuria — painful urination

Frequency — voiding >8×/day

Polyuria — excessive urination

Oliguria — diminished urination

Nocturia — urination at night

Hematuria — blood in urine

Anuria — no urine output

URINARY ALTERATIONS & DEVICES

- **Incontinence:** involuntary passage of urine; interventions — pelvic floor exercises, medications, limit caffeine/alcohol, skin protection
- **Retention:** bladder cannot fully empty; signs — inability to void, distention, hesitancy, weak stream; intervene based on cause
- **Bladder scan:** ultrasound to estimate post-void residual; reduces unnecessary catheterizations
- **External catheters:** condom catheter (male), Purewick (female); continuous suction 40–65 mmHg
- **Indwelling catheter:** sterile procedure; requires order; drain bag below bladder; empty when half full; assess for removal daily

BOWEL ALTERATIONS

- **Constipation:** <3 BM/week; hard dry stool; interventions — fluids, high fiber, stool softeners, suppositories, enemas, exercise
- **Impaction:** stool stuck in rectum from unrelieved constipation; may have liquid stool around the blockage; treat with enema or manual disimpaction
- **Obstruction:** partial or complete blockage — can be life-threatening; N/V, distention, severe constipation; may require NG tube or surgery
- **Diarrhea:** frequent loose/watery stool; monitor hydration; avoid irritants; Imodium
- **Ileus:** temporary loss of peristalsis; absent bowel sounds; NPO; common after abdominal surgery

C. DIFF

- Bacterial colon infection; risk factors: antibiotic use, age >65, weakened immune system, recent hospitalization
- Signs: watery, foul-smelling, yellow/mucous diarrhea; abdominal cramps; fever; rapid HR
- Complications: dehydration → kidney failure; sepsis
- **Contact precautions;** antibiotics (vancomycin or Flagyl); IV fluids; stool culture to confirm
- **Soap and water only for hand hygiene** — alcohol-based sanitizer does NOT kill C. diff spores

OSTOMIES

- **Ileostomy:** small intestine diverted through stoma; stool is liquid
- **Colostomy:** colon diverted; stool consistency depends on location (ascending = liquid; sigmoid = formed)
- **Stoma assessment:** should be moist, pink or red; blue = poor perfusion (call provider); black = tissue death (likely needs OR)
- Assess surrounding skin for breakdown with every bag change